NCE EXAM PREP

Practice Exam 02

Domain 2

Intake & Assessment

30 Practice Questions | 40 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 01. Use the suggested 40-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

If a condition is very uncommon in the population being assessed, what commonly happens to the positive predictive value of a screening test when other test characteristics remain constant?

A) It increases because false positives become less likely

B) It decreases because a larger proportion of positive results may be false positives

C) It becomes identical to the test's sensitivity

Question 2

> Before a skills group begins, a prospective member says, “I assume the counselor can guarantee that no member will ever repeat anything said here.”

Which response best supports informed consent for group counseling?

A) Confirm that the counselor can legally guarantee every member's confidentiality outside the group

B) Avoid discussing confidentiality because doing so may discourage participation

C) Explain the counselor's duties and the expectation that members protect privacy, while clarifying that other members' behavior cannot be guaranteed

Question 3

> During a couple's intake, one partner asks whether information disclosed in an individual check-in will always be kept secret from the other partner. The clinic uses a written no-secrets policy for couple counseling.

What should the counselor do before services begin?

A) Promise absolute confidentiality for every individual disclosure made by either partner

B) Wait until a private disclosure occurs before explaining how information will be handled

C) Explain the no-secrets policy, its limits, and how it applies to both partners, then obtain informed agreement

Question 4

> A recent immigrant seeks counseling for sleep disruption, headaches, and what the client calls “too many nerves” following exposure to community violence. The client does not initially use the word trauma and is unfamiliar with behavioral health screening.

Which assessment approach is most appropriate?

A) Explore the client's meaning of the symptoms, explain the purpose of screening, and use a suitable trauma measure alongside the interview

B) Interpret the phrase “too many nerves” as evidence of generalized anxiety and omit trauma questions to avoid discomfort

C) Use only the counselor's observations because standardized screening is inappropriate whenever a client uses culture-specific language

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Question 5

> A family requests counseling and brings a bilingual 12-year-old to interpret for a parent who has limited English proficiency. The family says using the child will save time, but the parent appears hesitant when sensitive topics are mentioned.

What is the best intake arrangement?

A) Use the child as interpreter because the family has already selected that role

B) Ask only yes-or-no questions so no interpreter is needed

C) Offer a qualified interpreter, explain interpreter confidentiality and roles, and obtain the parent's agreement about communication

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Question 6

> A counselor is beginning secure video services with a client who travels frequently within the counselor's service area. The client assumes that the video platform alone supplies everything needed for emergencies.

Which information should be addressed as part of telemental health intake?

A) Only the client's email address, because location information is unnecessary once identity is verified

B) A guarantee that technical interruptions will never occur during a session

C) How identity and current location will be confirmed, what to do after a connection failure, and how emergency support will be accessed

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Question 7

During an urgent assessment, a client states an intention to seriously harm a named former partner tonight, describes waiting near the person's workplace, and reports having a firearm in the car. The client refuses to agree to any immediate safety steps.

What should the counselor do next?

A) Initiate emergency safety procedures and a higher level of intervention while following applicable protective duties

B) Schedule a routine appointment for the next week because the threat has not yet been acted upon

C) Focus on a projective assessment to determine whether the statement symbolically represents anger

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Question 8

Twelve days after surviving a serious motor vehicle crash, a client reports intrusive memories, distressing dreams, flashbacks, negative mood, detachment, inability to recall part of the crash, avoidance of thoughts and driving, marked startle, and impaired work functioning. The symptoms began the day of the crash, and the client reports no previous trauma-related disorder.

Which diagnosis is most consistent with the duration and symptom pattern described?

A) Posttraumatic stress disorder

B) Acute stress disorder

C) Adjustment disorder with anxiety

Question 9

Before using a translated screening instrument with a client population, which evidence is most important to review?

A) Whether the translated version has the same number of words as the original

B) Whether reliability, validity, and norms are adequate for the intended language group and purpose

C) Whether the instrument is popular among counselors in unrelated settings

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Question 10

A Deaf adult requests an American Sign Language interpreter for an intake appointment. The client's hearing sibling offers to interpret and says this would be easier for the family.

What is the counselor's best response?

A) Arrange a qualified interpreter and discuss communication preferences directly with the client

B) Accept the sibling as interpreter because relatives are more familiar with the client's history

C) Conduct the intake through written notes without asking whether that method is effective for the client

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Question 11

A 72-year-old client presents with new confusion, slowed responses, and difficulty recalling the date. The client's adult child reports that these changes began two days after a primary care clinician adjusted several medications. The client was functioning independently the previous week.

What is the best assessment response by the counselor?

A) Diagnose a neurocognitive disorder based on the observed disorientation

B) Arrange prompt medical evaluation while assessing immediate safety and documenting the acute change

C) Complete a personality inventory to determine whether the confusion reflects psychological resistance

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Question 12

Two separated parents and their 13-year-old arrive for family counseling. Each parent assumes the counselor will keep a separate confidential file and advocate for that parent's preferred custody arrangement.

What should the counselor clarify before beginning services?

A) The parent who scheduled the appointment is automatically the only client

B) Who the client or clients are, the counselor's role, how records and information will be handled, and the limits of confidentiality

C) Family counseling requires the counselor to determine which parent should receive custody

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Question 13

A client's score is at the 84th percentile on an appropriately normed measure. What does this result mean?

A) The client answered 84% of the items correctly

B) The client's score is 84 standard deviations above the mean

C) The client's score equals or exceeds the scores of about 84% of the norm group

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Question 14

A 10-year-old is referred for possible attention-deficit/hyperactivity disorder. The caregiver reports chronic snoring and insufficient sleep, and the teacher notes that the child often seems not to hear instructions. No hearing or sleep evaluation has occurred.

Which assessment plan is best?

A) Gather symptom and impairment data across settings while evaluating developmental, hearing, sleep, and other possible contributors

B) Diagnose attention-deficit/hyperactivity disorder from the teacher's report because school impairment is sufficient by itself

C) Exclude attention-deficit/hyperactivity disorder entirely because snoring always accounts for inattentive behavior

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Question 15

A client with no history of mood episodes reports four days of very little sleep, unusually rapid speech, and increased activity. The symptoms began soon after a medical provider started a corticosteroid. The client denies prior similar periods.

What is the best next step in the diagnostic assessment?

A) Evaluate the temporal relationship to the medication and coordinate medical review before assigning a primary bipolar diagnosis

B) Diagnose bipolar I disorder because reduced sleep and rapid speech are sufficient to establish a manic episode

C) Diagnose insomnia disorder because sleep loss is the only symptom that requires assessment

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Question 16

For eight months, a client has experienced excessive, difficult-to-control worry about finances, family health, work performance, and routine errands. The client also reports restlessness, muscle tension, irritability, and disrupted sleep. The symptoms cause substantial distress and are not attributable to a substance or medical condition.

Which diagnosis best fits the information provided?

A) Generalized anxiety disorder

B) Adjustment disorder with anxiety

C) Panic disorder

Question 17

A university student attends an intake after a conduct office recommended counseling. The student says, “I was told I have no choice, and you will report whether I admit I caused the incident.” The referral document actually requests only attendance confirmation, and counseling participation is voluntary.

What should the counselor do?

A) Clarify the voluntary nature of counseling, the limited attendance disclosure, and the student's options before seeking consent

B) Begin questioning the student about the incident before correcting the misunderstanding

C) Tell the student that a conduct referral eliminates confidentiality for the entire counseling record

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Question 18

> An 8-year-old's teacher reports frequent inattention and incomplete work, while a caregiver reports focused behavior at home. A brief rating scale completed by each adult produces similarly discrepant results.

How should the counselor interpret the information?

A) Examine differences across settings and informants, obtain developmental and school data, and avoid drawing a conclusion from one reporter

B) Accept the teacher's rating as definitive because academic settings always provide the most valid behavioral observations

C) Average the two rating scores and diagnose solely from the resulting number

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Question 19

> An employee contacts a counselor after a supervisor recommended the agency's employee assistance program because of repeated lateness. The employee believes the counselor will send the supervisor a complete account of everything discussed.

What should the counselor clarify during intake?

A) The supervisor automatically becomes the counseling client because the supervisor initiated the referral

B) What information, if any, may be shared, the limits of confidentiality, and the employee's choices about participation

C) All session details must be released to the employer whenever work performance prompted the referral

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Question 20

Which mental status examination component is best illustrated by asking a client what the client would do after finding a stamped, addressed envelope on the sidewalk?

A) Remote memory

B) Fund of knowledge

C) Judgment

Question 21

> At a college counseling intake, a student reports recent superficial cutting and says, “Sometimes I wish I could disappear,” but adds that the cutting is not intended to cause death. The student appears calm and is willing to continue talking.

What is the counselor's most appropriate assessment response?

A) Assume there is no suicide risk because the student distinguishes cutting from a suicide attempt

B) Ask directly about suicidal thoughts, intent, plan, access to means, past behavior, and protective factors

C) End the interview and arrange involuntary hospitalization solely because any self-injury indicates imminent suicide risk

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Question 22

A counselor wants to know whether a measure produces similar scores when the same stable construct is assessed on two occasions. Which psychometric property is most directly relevant?

A) Content validity

B) Interrater agreement

C) Test-retest reliability

Question 23

> A client who uses a power wheelchair contacts a community counseling center. The office entrance is accessible, but the client reports that the standard intake room is difficult to navigate and asks whether the first meeting can occur by secure video instead.

Which intake response is most appropriate?

A) Ask about the client's access preferences, explain the available service formats, and confirm that the selected format can meet the client's needs

B) Schedule the standard intake room because all clients should complete the same intake process before accommodations are considered

C) Refer the client elsewhere immediately because a request for video intake indicates that in-person services are unsuitable

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Question 24

> A 15-year-old is brought to an outpatient clinic by a legal guardian. Clinic policy requires guardian consent for routine counseling and calls for the adolescent's assent when developmentally appropriate. The adolescent appears reluctant but is able to understand a plain-language description of services.

How should the counselor proceed with the intake?

A) Obtain only the guardian's signature because assent has no relevance when a minor cannot legally consent alone

B) Obtain the guardian's consent and invite the adolescent's informed assent, including discussion of privacy and its limits

C) Ask the adolescent to sign the adult consent form and exclude the guardian from all intake decisions

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Question 25

When the main purpose of an initial screening tool is to minimize missed cases of a potentially serious condition, which characteristic is most desirable?

A) High interrater reliability

B) High sensitivity

C) High specificity

Question 26

> At intake, a client reports receiving recent treatment from a primary care clinic and asks the counselor to speak with the medical provider. The counselor believes medication and laboratory information may be relevant to assessment.

What is the most appropriate way to obtain the collateral information?

A) Call the provider immediately because the client's verbal request authorizes disclosure of any medical information

B) Ask the client's relative to obtain the records so formal permission is unnecessary

C) Discuss the specific information and purpose with the client, obtain appropriate authorization, and request only relevant records

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Question 27

> A 46-year-old client seeking help for “panic” describes a first episode of chest pressure, shortness of breath, and dizziness that began while climbing stairs. The client has diabetes and has never received a medical evaluation for these symptoms.

What is the best assessment decision?

A) Confirm panic disorder because the symptoms occurred suddenly and were frightening

B) Administer only an anxiety inventory before considering any other explanation

C) Encourage prompt medical evaluation while continuing to assess the psychological features and context of the episode

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Question 28

> A 19-year-old reports severe dietary restriction and fainting. During assessment, the client has a resting heart rate of 42 beats per minute, appears weak, and reports being unable to keep fluids down since yesterday.

Which disposition is most appropriate?

A) Begin standard weekly outpatient counseling because motivation for change is the main level-of-care criterion

B) Arrange immediate medical evaluation because the findings suggest possible medical instability

C) Defer all action until a body image inventory has been scored

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Question 29

> During an initial interview, a client repeatedly looks toward the office door, speaks quietly, and pauses before answering. The counselor is documenting the mental status examination.

Which note best separates observation from interpretation?

A) “Client looked toward the door repeatedly and spoke softly; the reason for this behavior was not established.”

B) “Client was paranoid and expected an attacker to enter the office.”

C) “Client's guarded behavior confirmed a psychotic thought process.”

Question 30

> A client at an outpatient intake reports drinking heavily every day for several months and having a previous withdrawal seizure. The client stopped drinking yesterday and now has marked tremor, sweating, nausea, and increasing agitation.

Which level-of-care decision is most appropriate?

A) Begin routine weekly counseling and reassess withdrawal symptoms at the next visit

B) Refer for a nonurgent substance-use evaluation within the next several weeks

C) Facilitate immediate medical assessment for potentially dangerous alcohol withdrawal

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PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: B

Correct Answer: It decreases because a larger proportion of positive results may be false positives

Explanation: When prevalence is low, even a useful screening test may yield a lower proportion of true positives among all positive results. Positive predictive value depends partly on prevalence and is not interchangeable with sensitivity.

Question 2 — Correct: C

Correct Answer: Explain the counselor's duties and the expectation that members protect privacy, while clarifying that other members' behavior cannot be guaranteed

Explanation: Group informed consent should distinguish the counselor's confidentiality obligations from the conduct of other members. The counselor can establish expectations and discuss risks but cannot guarantee that every participant will preserve confidentiality.

Question 3 — Correct: C

Correct Answer: Explain the no-secrets policy, its limits, and how it applies to both partners, then obtain informed agreement

Explanation: The handling of individual disclosures is material to informed consent in couple counseling and should be explained before services begin. An absolute promise conflicts with the clinic's policy, while delaying the explanation prevents the partners from making an informed choice.

Question 4 — Correct: A

Correct Answer: Explore the client's meaning of the symptoms, explain the purpose of screening, and use a suitable trauma measure alongside the interview

Explanation: The counselor should understand the client's own idiom of distress and combine culturally responsive interviewing with an appropriate screening tool. Neither prematurely assigning a diagnosis nor rejecting all standardized measures provides an adequate assessment.

Question 5 — Correct: C

Correct Answer: Offer a qualified interpreter, explain interpreter confidentiality and roles, and obtain the parent's agreement about communication

Explanation: A qualified interpreter supports accuracy, privacy, and appropriate boundaries in a sensitive family intake. Placing a child in the interpreter role can inhibit disclosure and create role strain, while reducing the interview to yes-or-no questions would impair assessment.

Question 6 — Correct: C

Correct Answer: How identity and current location will be confirmed, what to do after a connection failure, and how emergency support will be accessed

Explanation: Telemental health consent should address location verification, technology disruptions, and emergency procedures. A platform does not replace an agreed contingency plan, and current location may be essential when urgent help is needed.

Question 7 — Correct: A

Correct Answer: Initiate emergency safety procedures and a higher level of intervention while following applicable protective duties

Explanation: The named target, stated intent, imminent timing, preparatory behavior, weapon access, and refusal of safety steps indicate an acute risk to another person. Routine follow-up or additional nonurgent testing would not adequately address the immediate danger.

Question 8 — Correct: B

Correct Answer: Acute stress disorder

Explanation: The trauma-related symptoms began after the crash and have persisted for 12 days, which falls within the duration window for acute stress disorder. Posttraumatic stress disorder requires symptoms lasting more than one month, while the described pattern is more specific than a general adjustment response.

Question 9 — Correct: B

Correct Answer: Whether reliability, validity, and norms are adequate for the intended language group and purpose

Explanation: Translation alone does not establish that scores have the intended meaning for a particular language group and use. Relevant evidence about reliability, validity, and normative interpretation is necessary before relying on the results.

Question 10 — Correct: A

Correct Answer: Arrange a qualified interpreter and discuss communication preferences directly with the client

Explanation: Communication access should be guided by the client and supported through a qualified interpreter when requested. Relying on a relative can compromise accuracy, privacy, and role boundaries, while written notes may not match the client's preferred or most effective communication method.

Question 11 — Correct: B

Correct Answer: Arrange prompt medical evaluation while assessing immediate safety and documenting the acute change

Explanation: An abrupt cognitive change following medication adjustments raises concern for a medical or medication-related condition and warrants prompt evaluation. A neurocognitive diagnosis should not be assigned from this brief presentation, and personality testing would not address the acute risk.

Question 12 — Correct: B

Correct Answer: Who the client or clients are, the counselor's role, how records and information will be handled, and the limits of confidentiality

Explanation: In family work, the parties need a shared understanding of who is receiving services and how information and records will be managed. The counselor should not assume that the scheduling parent is the sole client or accept an unstated custody-evaluation role.

Question 13 — Correct: C

Correct Answer: The client's score equals or exceeds the scores of about 84% of the norm group

Explanation: A percentile rank describes a score's relative position within the relevant norm group. It is not the percentage of items answered correctly and does not represent a number of standard deviations.

Question 14 — Correct: A

Correct Answer: Gather symptom and impairment data across settings while evaluating developmental, hearing, sleep, and other possible contributors

Explanation: Attention problems require evidence across contexts and careful consideration of alternative or co-occurring explanations. Sleep and hearing concerns are clinically relevant but neither automatically prove nor exclude attention-deficit/hyperactivity disorder.

Question 15 — Correct: A

Correct Answer: Evaluate the temporal relationship to the medication and coordinate medical review before assigning a primary bipolar diagnosis

Explanation: The close onset after corticosteroid exposure makes a medication-related explanation important in the differential diagnosis. The available facts are insufficient to assume a primary bipolar disorder, and the presentation includes more than isolated insomnia.

Question 16 — Correct: A

Correct Answer: Generalized anxiety disorder

Explanation: The client has excessive, difficult-to-control worry across multiple domains for longer than six months, with associated symptoms and significant distress. The pattern is not limited to a discrete stressor or recurrent unexpected panic attacks.

Question 17 — Correct: A

Correct Answer: Clarify the voluntary nature of counseling, the limited attendance disclosure, and the student's options before seeking consent

Explanation: The student cannot make an informed choice while holding inaccurate beliefs about coercion and disclosure. Correcting those beliefs before consent respects the actual referral terms and avoids implying that the entire record is open to the conduct office.

Question 18 — Correct: A

Correct Answer: Examine differences across settings and informants, obtain developmental and school data, and avoid drawing a conclusion from one reporter

Explanation: Discrepant reports may reflect real context differences, different expectations, or measurement limitations and should be investigated rather than erased. Diagnosis should integrate multiple sources and evidence of symptoms and impairment across relevant settings.

Question 19 — Correct: B

Correct Answer: What information, if any, may be shared, the limits of confidentiality, and the employee's choices about participation

Explanation: Referral by an employer does not by itself make the supervisor the client or authorize release of the full counseling record. Clarifying disclosure boundaries and participation expectations corrects the employee's misunderstanding before consent.

Question 20 — Correct: C

Correct Answer: Judgment

Explanation: The hypothetical asks how the client would respond to a practical situation and is commonly used to sample judgment. It does not primarily test recall of past events or breadth of acquired factual knowledge.

Question 21 — Correct: B

Correct Answer: Ask directly about suicidal thoughts, intent, plan, access to means, past behavior, and protective factors

Explanation: Nonsuicidal self-injury and a passive death wish require a direct, individualized suicide risk assessment even when the client denies suicidal intent for the cutting. The facts neither justify assuming no risk nor establish imminent danger requiring hospitalization without further assessment.

Question 22 — Correct: C

Correct Answer: Test-retest reliability

Explanation: Test-retest reliability estimates the consistency of scores across repeated administrations when the construct is expected to remain stable. Content validity concerns coverage of a construct, while interrater agreement concerns consistency between evaluators.

Question 23 — Correct: A

Correct Answer: Ask about the client's access preferences, explain the available service formats, and confirm that the selected format can meet the client's needs

Explanation: Effective intake identifies accessibility needs collaboratively and determines whether an available format is suitable. Automatically imposing the standard room or referring the client elsewhere would bypass the client's stated preferences and an available accommodation.

Question 24 — Correct: B

Correct Answer: Obtain the guardian's consent and invite the adolescent's informed assent, including discussion of privacy and its limits

Explanation: The stated policy requires guardian consent and developmentally appropriate assent. Including the adolescent in a clear discussion of services and privacy supports meaningful participation without misrepresenting the guardian's role.

Question 25 — Correct: B

Correct Answer: High sensitivity

Explanation: High sensitivity helps a screening tool identify people who have the condition and therefore reduces false negatives. Specificity instead concerns correctly identifying those who do not have the condition.

Question 26 — Correct: C

Correct Answer: Discuss the specific information and purpose with the client, obtain appropriate authorization, and request only relevant records

Explanation: The counselor should obtain informed, appropriately scoped permission and limit the request to information relevant to the assessment purpose. A general verbal request or use of a relative does not provide the same clear authorization and control over disclosure.

Question 27 — Correct: C

Correct Answer: Encourage prompt medical evaluation while continuing to assess the psychological features and context of the episode

Explanation: New exertional chest symptoms in a client with a medical risk factor require medical causes to be evaluated. Psychological assessment can continue, but the counselor should not assume panic disorder before potentially serious contributors are addressed.

Question 28 — Correct: B

Correct Answer: Arrange immediate medical evaluation because the findings suggest possible medical instability

Explanation: Severe restriction combined with syncope, marked bradycardia, weakness, and inability to retain fluids suggests possible medical instability. Assessment scores and routine outpatient scheduling should not delay urgent medical evaluation.

Question 29 — Correct: A

Correct Answer: “Client looked toward the door repeatedly and spoke softly; the reason for this behavior was not established.”

Explanation: This statement records directly observed behavior and appropriately acknowledges that its meaning has not been determined. The other choices convert limited observations into unsupported conclusions about paranoia or psychosis.

Question 30 — Correct: C

Correct Answer: Facilitate immediate medical assessment for potentially dangerous alcohol withdrawal

Explanation: A prior withdrawal seizure plus current autonomic and neurologic symptoms indicates risk of severe withdrawal. Routine or delayed outpatient follow-up would not match the potential medical urgency.
